



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Mental Health Assessment Unit – Safe Ward Environment

Reference #: FSH-MENH-POL-0011

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Mental Health Assessment (MHA) Unit	All staff

1. Introduction

Approximately three quarters of inpatients who die by suicide on psychiatric wards do so by hanging/strangulation¹. A significant proportion of suicides are believed to occur through impulsive acts using available ligature points. This document outlines the processes that are in place to maintain a safe patient environment and minimise risk on the MHA.

2. Terminology

Ligature Points	Any feature in an environment which could be used to support a noose or other strangulation device for the purpose of attempting suicide. Ligature points may include: doors, windows, cupboards, wardrobes, sinks, bath / sink taps, exposed pipes, fire extinguishers, curtains blinds / rails, ceiling extractor fans, radiators, fire alarms, toilet cistern / toilet roll / soap, paper towel dispensers, light fittings, shower head / control shower cubicle doors / curtain rails.
Unsafe items	Unsafe items may include medications, alcohol and other drugs, sharp instruments, knives, razors, glassware, pins, spray deodorants, hair ties, plastic bags, matches, lighters, belts, ties, shoelaces, electrical cables / cords, gym equipment (e.g. dumbbells).

3. Policy

- Ligature points will be, where possible, removed or covered.
- Where it is not possible to remove or cover all potential ligature points, risk mitigation and management strategies must be in place.

- Patients are only allowed access to areas of the MHA where known ligature risk points have been identified when staff member/s are available to supervise or in the company of visitors. These areas include:
 - central family room
 - comfort/quiet room
 - interview rooms
 - Courtyard 2.

All staff must be made aware of any blind spots within the unit. The use of monitoring equipment or mirrors to reduce this risk must be considered.

- Access to unsafe/potentially harmful items will be restricted to ensure the safety of patients and staff. Access, reconciliation and storage of these items will be managed by MHA staff.

4. Procedure

4.1. Admission to ward

- On admission staff will ask patients to allow them to examine any belongings brought with them, including items in their clothing to prevent unsafe items being carried onto the unit. Refer to the [SMHS Patient search and identification of dangerous items](#) policy.
 - The clinical rationale for the removal of items e.g. individual risk, ward environment must be documented in the patient's medical record and where appropriate explained to the patient.
 - Patient's own medications (prescribed and over-the-counter) must be stored in the clean utility room.
 - A review of the patients own medications will occur during the admission by the treating team and pharmacist wherever possible. A decision will be made regarding those medications that are safe to return to the patient on discharge.
 - Inpatients will be provided with a secure place to store their personal items / property when reasonably practical.
 - Items considered to present a risk must be stored in the patient storeroom.
 - Items that are deemed to be safe can be stored in the patient's room.
 - All patient property brought into the mental health unit must be clearly documented on the Patient Property form. A copy of the form must be given to the patient.

4.2. Risk assessment and management

- All patients will have a clinical risk assessment completed on admission and at regular review. This will include the risk to patient in relation to unsafe / potentially harmful items.
- The medical and risk assessment will inform the individual patient's observation requirements and inform the development of a Treatment Support and Discharge Plan in collaboration with the patient and their family/significant other (where possible).

4.3. Ward environmental checks

- Environmental checks of the ward and courtyards by the Shift Coordinator (or delegate) will be conducted at the commencement of each shift and documented on the environmental checklist.
- Patients will be accompanied by staff at all times in areas with unsafe/potentially harmful items e.g. family and interview rooms, comfort/quiet room, kitchen and Courtyard 2 and these areas will be locked when not in use.
- All equipment in these areas will be reconciled by staff after each therapy session or daily. Equipment will then be locked in the provided cupboards after use.
- The Activity of Daily Living (ADL) kitchen area is provided on the Youth Unit. The ADL kitchen is only for patients and no additional equipment should be brought into this area e.g. china plates, glasses.
- Any kitchen equipment required for functional assessment or groups that may be deemed unsafe or / potentially harmful items (such as sharp knives) will be provided by staff as required.
- Patients must be supervised during use of these items. After use, the equipment will be collected and reconciled by staff.
- Sharps are signed for in the sharps register and locked in the drawer provided between uses.

4.4. Visitors

- Items brought into MHA by visitors for patients must be inspected by the patient's nurse to ensure there are no unsafe items that may pose a risk on the ward and will be documented on a new Patient Property form and added to the original.
- Lockers will be provided to store visitor belongings if required.

5.Compliance/Performance Monitoring

The Nurse Unit Manager will be responsible for monitoring compliance against this document. Compliance will be evaluated via:

- routine clinical incident review processes
- completion and action of an annual “Access to means of self-harm audit”

6.Related Policy Documents

SMHS:

- [Patient search and identification of dangerous items](#)
- [Rights of patients in Mental Health](#)

FSFHG:

- Patient Observation - Specialising and Close Observation (FSFH-MENH-POL-0023)
- Courtyard access and observations (FSFH-MENH-POL-0005)
- CCTV monitoring in Mental Health Clinical areas (FSFH-MENH-POL-0011)
- Sensory rooms and sensory equipment: safe and therapeutic use (FSFH-MENH-GUI-0006)
- Sexual Safety in Mental Health (FSFH-MENH-POL-0012)
- Visiting in Mental Health Inpatient areas (FSFH-MENH-POL-0016)
- Code Black in the Mental Health Unit (FSFH-MENH-POL-0014)
- Sharp items: safe handling during therapeutic activities (FSFH-MENH-POL-0024)
- Clinical Risk: Assessment and Management (FSFH-HW-POL-0017)

7.Related Standards

NSQHS:

- Clinical Governance
- Comprehensive Care

Chief Psychiatrists Standard's for Clinical Care:

- Risk assessment

8.Related documents

[The Mental Health Act 2014](#)

Record of Patient Property Form

9.References

- 1.Hunt IM, Windfuhr K, Shaw J et al. 2012. Ligature points and ligature types used by psychiatric inpatients who die by hanging: a national study.
<https://www.ncbi.nlm.nih.gov/pubmed/22343063> Accessed 04/07/2019

10. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	18.6.20	Nurse Unit Manager, Mental Health Assessment Unit	S5 SQR Committee	Policy Committee	11/2018
2	06/2022	Nurse Unit Manager, Mental Health Assessment Unit	NMPGC	S5 SQR Committee	06/2024
V3	08/2024	Nurse Unit Manager, Mental Health Assessment Unit		S5 SQR Committee	09/2027